1. AFRICA ALBIDA TOURISM (PVT) LTD **HC1800/22**

And

VICTORIA FALLS SAFARI LODGE HOTEL (PVT) LTD

Versus

ZIMNAT GENERAL INSURANCE

2. SPENCERS CREEK (PVT) LTD **HC1801/22**

Versus

ZIMNAT GENERAL INSURANCE

HIGH COURT OF ZIMBABWE
MPOKISENG DUBE J
BULAWAYO, 3, 4, 27 FEBRUARY & 05 MAY 2026

Civil Trial- Insurance claim

Adv D.Tivadar for the plaintiffs
Adv T.R Mafukidze for the defendant

MPOKISENG DUBE J: I have previously delivered judgment in this matter in an application for absolution from the instance under judgement number HB122/25. For that reason, I shall not regurgitate the facts that had been placed on record up to that point.

As I have alluded in that judgment the COVID 19 virus took the world by storm and completely turned livelihoods and the world of commerce on its head. A matter such as this would have been a perfect test case for putting into effect the provisions of Section 174 (d) of our Constitution (Constitution of Zimbabwe Amendment (No. 20) Act, 2013). It provides as follows:

> "174 Other courts and tribunals
>
> An Act of Parliament may provide for the establishment, composition and jurisdiction of –
> …….

(d) tribunals for arbitration, mediation and other forms of alternative dispute resolution."(My own underlining for emphasis)

This Constitutional provision puts to the fore the often less thought of mechanisms of alternative dispute resolution. Besides the obvious advantages of being cost effective, offering faster resolution times, greater privacy, more flexible, tailored solutions, the process is inherently conciliatory and preserves relationships. (visit ADR Institute of Canada; https//adric.ca)

I am of the considered view that had the parties herein taken such route they could have walked away with several obvious benefits some of which could have been preservation of cordial client-service provider relations and go through a less intrusive public probing of each other's *uberrima fides*. I have no doubt in my mind that both sides are key role players in the national fiscus.

Be that as it may the Rubicon has long been crossed.

Now that both parties have placed their evidence before court, I am now at liberty to analyse and comment in depth on every piece of evidence I find relevant.

In re AFRICA ALBIDA TOURISM (PVT) LTD and VICTORIA FALLS SAFARI LODGE HOTEL (PVT) LTD HC 1800/22

It is common cause that the 2nd plaintiff being a subsidiary of the 1st jointly held an insurance policy in respect of their tourism business, with defendant under policy number T4/.ASS/USP/009876 for the period 1st April 2019 to 1st April 2020. Specifically, the claim arises from Clause 5.8 within the 'Loss of Revenue Section' of the insurance contract. The clause provides coverage to the plaintiffs against business interruption that results from the occurrence of a "notifiable contagious or infectious disease" at their business premises. They seek payment in the sum of US$ 1. 066, 239.00 under the insurance cover.

In re SPENCERS CREEK (PVT) LTD HC 1801/22

Plaintiff held an insurance policy in respect of its premises, being Ilala Lodge, with defendant under policy number T4/.ASS/USP/00145703 for the period 1August 2019 to 1 August 2020. The plaintiff seeks payment of the sum of US$ 1. 400, 000.00 under the said cover.

The background facts of the matter are common cause. The plaintiffs pleaded that from March 7 to March 10, 2020, two elderly guests, identified as Mr. and Mrs. O'Shaugnessy, were accommodated at Ilala Lodge, Victoria Falls. During their stay, they reportedly contracted COVID-19, exhibited certain symptoms, sought medical attention around March 7, 2020, and subsequently tested positive for the virus. I must point out that during the trial it emerged that this couple was unwilling to testify. Whether they subsequently tested positive for the virus was then not proven with certainty.

The plaintiffs pleaded that in an effort to contain the COVID-19 outbreak and in compliance with a national directive issued by the President of the Republic of Zimbabwe on March 27, 2020, the plaintiffs assert they were compelled to close their business premises. The plaintiffs claim that this mandatory closure lasted for at least three months from March 30, 2020. They claim that as a direct consequence of this business interruption and closure, the plaintiffs allege a total loss of income amounting to US$ 1,066,239.00 for the matter under HC 1800/22 and US$1, 4000.000.00 for the matter under HC 1801/22.

The plaintiffs led evidence from their various employees who detailed a chain of contacts. All the persons in that chain of contacts displayed certain symptoms which plaintiff's aver were COVID 19 symptoms. In brief that; the O'Shaughnessys were in contact with employees of Ilala Lodge, which led to six of Ilala Lodge's employees, including Jacob Mapetere, developing COVID-19 like symptoms around the same period. In general they all felt flue like symptoms. They felt weak, nauseous, with chest pains, a severe head ache, a cough and in most cases with a runny nose. Subsequently, Jacob Mapetere allegedly came into contact with two of the plaintiffs' own employees, Lin Jones Pullen and Ovius Ngwenya. Shortly after this encounter, both Lin Jones Pullen and Ovius Ngwenya also developed COVID-19 like symptoms. The narrative continues, stating that these two plaintiffs' employees, in turn, had contact with other employees of the plaintiffs, who also subsequently developed COVID-19 like symptoms. The plaintiffs give a detailed account of the infection chain, from guests to Ilala Lodge employees and then to the plaintiffs' own staff. Some of the employees sought medical attention from local private doctors while some resorted to home remedies. Notably none were tested for the COVID 19 virus.

It is important to note that the O'Shaughnessy couple were in the country from the 7th -10th March 2020. All the plaintiff's employees fell sick during this period but recovered with a space of 14 days or so. The very first COVID case in Zimbabwe was discovered on the 20th

March of the same year. Prior to this date and per the later testimony of Dr Jeans, there were no set mechanisms to control the spread of COVID. Compulsory testing of suspected tests was not yet in place.

The plaintiffs led the evidence of Desmond Gerald Stevens. He testified that he is an insurance broker by profession in the tourism industry. All three plaintiffs are his clients. They had taken out policies with the defendant covering business interruption due to an occurrence of a notifiable disease at their premises. COVID 19 is a notifiable disease. According to this witness this notifiable disease occurred at his clients' premises. Their premiums were all paid up. He duly notified the defendant through its Ms Chigayo on the 23rd March 2020. The email he wrote reads as follows:

> "*There are no doubt going to be claims arising out of the outbreak of the above disease. Whilst we are at this stage unsure of exactly how Insurers are going to respond, there will be a certain amount of cover particularly where clients have Business Interruption cover. Please accept this as official notification for all our clients that they have suffered a reduction in their Revenue/turnover as a result of COVID-19. Our clients will be putting together their claims as we progress but this will be a drawn-out exercise. In order to comply with the 30-day notification clause, please accept this as being official notification.*
>
> *I look forward to receiving your confirmation of receipt and that this notification has been formally accepted."*
>
> To this Ms Chigayo replied;
>
> "*Your email is acknowledged with thanks.*
>
> *For purposes of notifying our Re-insurers and in line with our best practice for loss reserving, may we have specific individual details of affected clients and circumstances culminating into BI loss."*

From the above exchange one can clearly deduct the following:

a) Mr Stevens is specifically identifying this email as official notification of claims by the plaintiffs.
b) The claims would be for Business Interruption (B.I) as covered by their polices.
c) This is meant to comply with the 30-day notification clause of the policy.

d) He implores Ms Chigayo to confirm receipt of the notice and to indicate whether or not it has been accepted as such.

In her reply Ms Chigayo does not mention that the notification was invalid. If anything, she is thankful for the email. For her purposes she raises no query. She only indicates that

"*For purposes of notifying the Re-Insurers and ..... for purposes of "loss reserving*" she needs specific individual details of the affected clients and their circumstances.

My understanding of this exchange is that the notification is accepted as sufficient and valid. Extra and specific info is required only for purses of Re-Insurers. I therefore reject Ms Chigayo's testimony in court that she rejected the notice as being invalid. This surely was not expressed in her reply to Mr Stevens' email which was very precise on its purpose.

Further to that, on the 2nd April 2020 one Kumbirai Chitepo from the Claims Department, addressed an email to Mr Stevens copied to Ms Chigayo to the following effect;

"*Hie Des*

Our claims team will be sending the relevant claim form shortly"

Why would the claims team send a claim form if the notice given was defective? Be that as it may he was given a form to complete in respect of each plaintiff. He protested that the form was not correct as it related to "Fire and Allied Perils". Mr Stevens specifically queried *"Is this for Business Interruption claim*?" per email of 2nd April sent to Levy Njanje. To which he replied:

"We do not have a specific BI claim form but the all risks is the one that we use for such losses"

Defendant's Claims Department personnel insisted that it was the correct form. I must comment that clearly the form was not correct. The form is entitled "Fire, Lightning, Storm Claim Form." Specific perils that the form is relevant to are listed individually. It is Fire, Lightning and Storm. In no way can this form cover "an occurrence of a contagious disease" A disease is not related to a fire, lightning nor a storm. The form goes on to require information which is completely vague and unrelated to occurrence of a contagious disease. For example it requires:

1. *Date of loss …….* *Time……. Cause…..*
2. *Where did it occur?...*
3. *Where can damaged property be inspected?*
4. *Is damaged property insured with any other office?*
5. *Has any other person or company an interest in the damaged property?*
6. *Pleas give full circumstances leading up to the damage.*

Clearly para 4 to 6 relates to corporeal items. So is para 7- 12 which fall under the sub-heading "*Please complete in respect of electrical equipment claims*". Out of 12 questions asked only paragraphs 1-2 appear relevant. Mathematically it means this form is 16.7% relevant, that is less than 20% applicable to the claim.

I am of the respectful view that this claim form was never designed for a claim such as the present. The reason is revealed in a letter addressed to Mr Stevens by Ms Chigayo dated 10 July 2020. At paragraph 1 line 3 it reads;

> "*In this previously unimaginable environment, the medical and social challenges are great and the difficulties facing every business unprecedented."*

What this letter tells in plain truth is that the defendant in its position as an insurer was taken by surprise by the pandemic and resultant claims "previously unimaginable" as in the words of Ms Chigayo. Indeed, everyone was caught unawares by this devastating pandemic and its destructive force. Now out of this vague and ill-fitting claim form the defendant cannot expect absolute perfection from their clients in its completion.

The defendant needless to say responded on the 9th of June effectively refusing to honour the claims. When Ms Chigayo later testified she said she repudiated plaintiff's claims based on the claim form they submitted and the policy document. As I have already stated, it can not be proper to reject a claim by the insured client based on a vague and irrelevant form. Ms Chigayo said they also relied on investigations done by Millennium Risk Consultants. That cannot be correct as per the testimony of Mr Chakauya who was tasked to investigate if COVID 19 had occurred at plaintiff's premises a year after the event. That was long after the plaintiff's claims had been rejected.

The plaintiffs further called a medical expert witness a Dr M.D. Jeans who stated that he is a medical doctor with a post graduate degree in Tropical diseases. He stated that at the time relevant to these claims he was working as a medical doctor in Victoria Falls. He was the

first person to diagnose the very first COVID 19 case in Zimbabwe before he went on to diagnose many others. At some point he was appointed to be the lead doctor for the COVID team in Victoria Falls. He was present in court when the employees of the plaintiff testified. He testified to the effect that in his opinion on a balance of probabilities they probably had contracted COVID 19. He said he drew his conclusion from the fact that the O'Shaughnessy couple met the classifications of a COVID 19 case. They had a reported COVID exposure, they came from an area in the UK which was experiencing an on going transmission of COVID. They were in contact with the employees of the plaintiff via a chain of contacts. He went on to comment that this couple fitted what he termed the Clinical Criterion as they had symptoms similar to those of COVID as well as the signs picked by the doctors who attended to them. He stated that finally they presented what he termed the Presumptive Laboratory Analysis being a positive anti body test upon their return to the UK.

I must pose here to state that when this witness testified the plaintiff's still hoped to avail the O'Shaughnessy couple. For that reason, he stated as a matter of fact that this couple had a positive anti body test upon their return to the UK. When the couple refused to co-operate this piece of evidence remains uncorroborated and cannot be accepted as a matter of fact. It is therefore expunged from the record as hearsay.

As for the employees Dr Jeans stated that even though they did not test for COVID they do fulfil the exposure criteria within the 14-day exposure period and they developed symptoms consistent with COVID19 infection as the timing was outside the usual influenza season in both the UK and Zimbabwe. The witness then said some people could carry the COVID virus without showing any symptoms. He further stated that at the time people could not be immediately tested as there was no laboratory in Victoria falls that could carry out COVID diagnostic tests. However, the treatment they received for those who sought it, was consistent with COVID management at the time.

I must also comment that it was not disputed by the defendant's counsel that at the time of the O'Shaughnessy's visit there were no laboratories in Victoria Falls and Hwange to carry out COVID diagnostic tests. I accept this position as the Covid outbreak was only declared a global pandemic by the World Health Organisation on the 11th March 2020. In Zimbabwe the government only made such declaration on the 23rd March 2020 per *SI 76 of 2020 (Civil Protection (Declaration of State of Disaster: Rural and Urban Areas of Zimbabwe) (*COVID-19) Notice, 2020.

This witness was also quite extensively cross-examined. His findings were compared to those of the doctors who personally examined the O'Shaughnessy couple. These were Dr D. Ndikudze of Chinotimba Medical Centre who assessed the couple on the 8th March 2020 and Dr F. Musinami-Mvura the District Medical Officer for Hwange who assessed the couple on the 9th March and cleared them as free from Covid and cleared them to travel back to their country of origin. It came to light that both these doctors did not carry out COVID tests but relied on interviewing the patients.

During the cross examination of Dr Jeans, it was put to him that he lacks experience as he only testified as an expert witness for the first time in this court. It was put to him that his conclusions are unreasonable as the symptoms he described can be attributed to other ailments such as malaria, the side effects of a malaria drug malof or just a bout of the common influenza virus. I found this witness credible by the close of the plaintiff's case.

I will at this juncture compare the testimony of Dr Jeans with the testimony of the defendant's expert Dr Knowledge Chemayi. He stated that he is a holder of a Bachelor of Medicine degree from the University of Zimbabwe, qualified for a fellowship programme in South Africa. He was at the time of his testimony employed at United Bulawayo Hospitals (UBH) and seconded to Ekusileni Infectious Diseases Hospital under the National University of Science and Technology (NUST). He runs his own medical practice. He joined UBH in year 2018. He stated that at Ekusileni he was tasked with COVID-19 management. He was deployed there in October 2020. His duties were to diagnose, manage and treat COVID patients. He would as part of his duties receive patients from other centres already diagnosed with COVID.

He was taken through the medical reports of both Drs Ndikudze and Musinami-Mvura. He interpreted the report by Dr Ndikudze to mean the patient Veronica Mary Frances O'Shaughnessy had a respiratory infection for one day. He said the symptoms suggested pulmonary embolism. He says this is consistent with someone elderly who travelled long distances and developed blood clots in the legs which travelled to the lungs.

He also argued that the month of March in the UK is end of winter and the conditions there are favourable for flue.

He commented further that Victoria Falls is a 'hot' malaria zone moreso the patient was taking malaria medications. He observed that malaria can also present similar symptoms. He

said it was crucial to check their contacts and that if they had no positive contact then there was a low possibility that the patient had COVID.

He observed further that the malaria prophylaxis Melaron can give side effects such as a dry mouth. He noted that Dr Ndikudze's report was erroneous possibly attributable to a typo as it showed a Respiratory Rate (R/R) of 266 which he says is impossible. He commented that the usual is 16-24 or up to 26 depending on the altitude where one resides.

He noted that all symptoms were in keeping with someone with a fever. He commented that on the comment, "*Patient is not keen to go to hospital and opted for oral antibiotics*" He said this could mean the doctor did not find it necessary to admit, otherwise a patient can not elect not to be admitted if it was crucial. He concluded that it meant their symptoms were not severe.

He noted that as part of the patient management plan Dr Ndikudze did COVID-19 Screening. He noted that at that time in March 2020 the screening of it comprised just checking for symptoms and ruling out the possibility of COVID.

He commented that in the post March 2020 era screening entailed taking samples for laboratory testing.

In the present case the patient was given Doxycycline which is malaria treatment and Azithromycin which is an antibiotic for treating chest infections. He notes that Dr Ndikudze could not have prescribed bacterial medicines as he did if he suspected a viral infection. He then reported to the District Medical Officer who in turn cleared the patient. He concluded that per Exhibit "S" ie the DMO's report it meant the patient was responding to medication which in turn indicate that the treatment was proper and effective.

Notably Dr Chemayi observed that COVID-19 can enter the body without making one sick. Its incubation takes 5-14 days but typically 5-7 days. He quipped that chances are we all came in contact with COVID patients and never fell ill. Further that COVID has always been with us without making us ill. He said as of now, COVID testing involve nasal swabs to test for antigens not blood samples.

He commented that from his observations all employees of the plaintiff's might have suffered from an influenza virus. He says looking at how doctors treated those that sought medical treatment and how they recovered he rules out COVID. He said that they all reacted positively to antibiotics and home remedies. COVID is a viral infection and does not react to

antibiotics. Further that from the symptoms one cannot tell COVID apart from flue but the treatment the patients received suggests an exclusion of COVID.

This witness was equally placed under intense cross-examination. He was accused of playing to the script. Particularly that when defendant filed its synopsis of evidence sometime in February 2024 it stated what he is going to testify in this trial yet per his own testimony he was only instructed to testify sometime 3 weeks before Dr Jeans testified in November 2024. It was put to him that in the synopsis of evidence he was said to be coming specifically to rebut evidence that COVID-19 ever occurred at plaintiff's premises. He confirmed that by March 7-10 there were no COVID testing facilities in Victoria Falls. He agreed that Dr Jeans having led the COVID task Team in Victoria Falls and having diagnosed the first COVID case in that same town, he was more eminently placed to comment on COVID related matters. He particularly said he accepts this due to Dr Jeans' qualifications and experience. He instead specialized in Gynaecology and Obstetrics. He however averred that due to his experience in managing patients on the ground, he had better insight than just qualifications on paper, and regardless of his specialization.

Dr Chemayi accepted that for one to come up with a conclusion one does not look at one piece of evidence but holistically at all other evidence. In *casu* he admitted that he did not check the UK community transmission chart (Epidemiological Chart) ie the histogram that visually represents the onset and progression of a disease outbreak over time. For that reason, he admitted that he is not holistically able to comment on the O'Shaughnessy case.

What caught my attention from Dr Chemayi's testimony is that he does not specialise in tropical diseases even though he has practical knowledge of its management. It is worrying that by February 2024 when defendant filed its synopsis of evidence, it already knew what he was coming to tell the court even before he was consulted. That was way before he was presented with the synopsis of evidence of plaintiff's employees.

From the sum total of Dr Chemayi's testimony he says Mrs O'Shaughnessy suffered from pulmonary embolism. This condition is not infectious. If indeed that was the case why did the people she came in contact with fall sick? If it was malaria, it is equally incapable of being transmitted via contact. I am convinced that Dr Chemai indeed played per the script presented to him. I thus reject his testimony as being biased.

I have already commented on the testimonies of Ms Chigayo, Mr Chakauya and that of Dr Chemayi.

On their closing submissions both counsel agree that plaintiffs' *causa* was based on an insurance contract. The contract required that there should be proof that a notifiable disease occurred at the insured premises on a balance of probabilities. If not the claims should fail. They both agree that for the claim to succeed it should be proven that the *causa* occurred at a particular time and at the particular premises owned by the plaintiffs. Counsel for the defendant however strongly contended that if the plaintiffs cannot prove that a notifiable disease occurred at either Ilala Lodge or Spencer's Creek then their claims must fail.

Counsel for the defendant argued quite forcefully that the insurance cover is for the occurrence of COVID 19 not its symptoms. That the symptoms shown by the elderly tourists might as well have been side effects of a malaria prophylaxis *malof* which they were confirmed to be taking.

He argued further that the notice to bring the claim was also made unreasonably out of time. That way it deprived the defendant an opportunity to carry out its independent investigations and to guard against a fraudulent claim. He contends that the plaintiffs and their broker were confused as to the type of their cover. They thought it was for reduction of tourist arrivals yet in actual fact they were covered for the occurrence of a notifiable disease at their premises at the time of their cover. For that reason, that reduced turnover was not the covered peril. He argued further that when the plaintiffs came to grips with the actual insured peril, they attempted to *post facto* adjust events and facts to suit the covered peril.

Counsel further attacked the quantum claimed as lacking authenticity from the source documents. Counsel for the defendant does not give alternative figures that the court may work with. He basically argues that Mr Matsika's calculations are not authentic and must be rejected.

As agreed the onus is on plaintiff's to prove that COVID19 happened at their premises. Such proof I accept is on a balance of probabilities as opposed to proof beyond reasonable doubt. From the 8 witnesses who testified for the plaintiff they had Flue/ COVID like symptoms. There is no proof that they had any other disease such as malaria. There is no proof that they had just a common flue. The expert witness called testified under oath to the effect that they most likely had COVID 19 per Dr Jeans or that they had all other diseases except COVID per Dr Chemayi. The expert evidence is mutually destructive. As indicted above

considering that there were no COVID -19 test facilities then all experts rely on the historical background of the symptoms. No actual tests were conducted to rule out COVID-19.

I agree with the case law authority cited by Counsel for the defendants in *Stellenbosch Farmers' Winery Group Ltd and Another v Martell* & CIE SA 429/2002 in which the Supreme Court of Appeal of South Africa stated that a court must assess the credibility of witnesses, reliability of witnesses and probabilities. In the light of the court's assessment of credibility, reliability and the probabilities the court will then, as a final step, determine whether the party burdened with the onus of proof has succeeded in discharging it.

I also read the matter of *National Employer's General Insurance v Jagers* 1984 (SA) 432 in which the court stated that ordinarily the party who bears the onus can discharge it only if that party has adduced credible evidence, particularly where there are mutually destructive versions.

Very often, the assessment of the witnesses and the general probabilities will be decisive. It was held as follows:

> 'It seems to me, with respect, that in any civil case, as in any criminal case, the onus can ordinarily only be discharged by adducing credible evidence to support the case of the party on whom the onus rests. In a civil case the onus is obviously not as heavy as it is in a criminal case, but nevertheless where the onus rests on the plaintiff as in the present case, and where there are two mutually destructive stories, he can only succeed if he satisfies the Court on a preponderance of probabilities that his version is true and accurate and therefore acceptable, and that the other version advanced by the defendant is therefore false or mistaken and falls to be rejected. In deciding whether that evidence is true or not the Court will weigh up and test the plaintiff's allegations against the general probabilities. The estimate of the credibility of a witness will therefore be inextricably bound up with a consideration of the probabilities of the case and, if the balance of probabilities favours the plaintiff, then the Court will accept his version as being probably true. If however the probabilities are evenly balanced in the sense that they do not favour the plaintiff's case any more than they do the defendant's, the plaintiff can only succeed if the Court nevertheless believes him and is satisfied that his evidence is true and that the defendant's version is false."
>
> (See also *Joadaan v Koekemoer and Anor* CA52-2010)

Applying the following dictum to the present matter on a balance of probabilities, I am convinced that indeed the O'Shaughnessy couple suffered from COVID-19. Their presence in court could have proven or rebutted each side's case beyond reasonable doubt. Their absence however does not affect the test of proof on a balance of probabilities. It is on record that they expressly refused to testify when counsel for the plaintiff made efforts to avail them. I cannot

visit any party with negative inferences for failing to avail them. I am convinced that Dr Jeans testified to the best of his abilities without bias. Similarly I am convinced that Mr Stevens did his utmost to notify the defendants on time. It is the defendants who were ill prepared. They supplied him with the wrong form. He did his utmost to complete the form as truthfully as he could but it was vague. The defendants now want to place the blame on him for supplying them with incorrect info, which is unfair.

I am convinced that plaintiff's business was interrupted due to the occurrence of COVID-19 at their premises per clause 5.8(b) of the policy (page 23). While the loss of business was not directly related to the O'Shaughnessy visit, the national lock down was as a result of the occurrence of a contagious disease. It resulted in losses as claimed. I have no alternative figures to work with than those presented by Mr Matsika for the plaintiffs.

In the final I am persuaded that the plaintiffs made out their case for the relief they seek.

Disposition

In the foregoing, the following order is made under HC 1800/22:

1. The defendant be and is hereby ordered to pay the sum of US$1 066 239. 00 (one million and sixty-six thousand two hundred and thirty- nine United States Dollars), being monies due to the plaintiffs in terms of the insurance contract between plaintiffs and defendant;

2. Interest on the above-mentioned amount at a rate of 5% per annum from the date of service of the summons to date of full payment;

3. Costs of suit.

The following order is made under HC 1801/22

1. The defendant be and is hereby ordered to pay the sum of US$1 400 000. 00 (one million four hundred thousand United States Dollars), being monies due to the plaintiff in terms of the insurance contract entered into between plaintiff and defendant;

2. Interest on the above mentioned amount at a rate of 5% annum from the date of service of the summons to date of full payment;

3. Costs of suit.

Webb Low & Barry Inc. Ben Barons & Partners- plaintiff's legal practitioners
Atherstone & Cook- defendant's legal practitioners